

RxKids Hawai'i - Cost & Benefits by Income Quintile, TY 2028

Statutory eligibility: Medicaid (clause 1) OR income <= 300% FPL incl. expected unborn child (clause 2)

Point estimate - true uncertainty is +/-30-40%. No admin caseload exists to calibrate against; cost hinges on soft pregnancy/infant incidence assumptions. Read with the assumption band.

Fiscal cost (annual)

Total program cost (benefit)	\$32,087,983
Prenatal arm	\$10,109,912
Postnatal arm	\$21,978,071
Administrative load (8%)	\$2,567,039
Appropriation total (benefit + admin)	\$34,655,022
Sampling 90% CI	\$28,633,408 - \$35,542,559
Assumption band	\$19,252,790 - \$40,912,789

Scenario comparison

Take-up held constant (0.90 newborn / 0.83 prenatal); only the eligibility gate and postnatal window change. Cost = annual benefit dollars; appropriation adds the admin load.

Statutory (Medicaid OR <=300% FPL) · 6-mo postnatal	\$4,500	\$32,087,983	\$34,655,022	14,066
Universal · 6-mo postnatal	\$4,500	\$49,801,763	\$53,785,904	21,831
Universal · 12-mo postnatal	\$7,500	\$83,912,559	\$90,625,564	21,831

Potential option - extend postnatal 6-12 months

The program may opt to extend postnatal payments to the full 12 months (Flint's upper bound). Priced here as an optional add-on:

Additional cost (+6 months)	\$21,978,071
Total cost (12-month design)	\$54,066,054

First fiscal year (launch) - 12mo operating, 12mo enrollment ramp

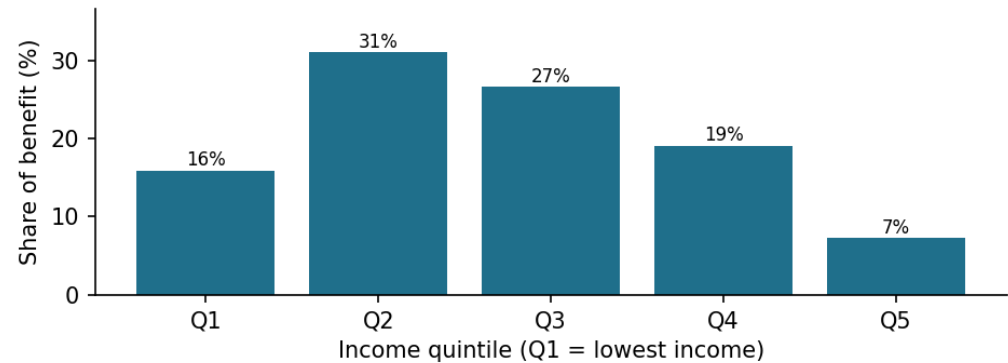
Year-1 cash disbursement, below steady state: enrollment ramps up and the 6-month postnatal caseload takes time to fill.

Year-1 disbursement (total)	\$13,310,978
Prenatal / Postnatal	\$5,476,203 / \$7,834,775
Postnatal deferred to next FY	\$4,070,013
Year-1 as % of steady state	41%
Ramp sensitivity (6 / 12 / 18 mo)	\$20,824,222 / \$13,310,978 / \$8,873,985

Household reach

Eligible families (weighted)	8,158
Expected recipients / year	14,066
Expected pregnancies / infants	6,740 / 7,326
Avg benefit per recipient	\$2,281

Benefit received by income quintile



Quintile	Avg income	Families	Recipients	Total benefit	Share
Q1	\$15,315	107,149	2,230	\$5,087,073	15.9%
Q2	\$52,619	107,150	4,377	\$9,984,613	31.1%
Q3	\$92,200	107,159	3,745	\$8,544,160	26.6%
Q4	\$146,436	107,149	2,690	\$6,137,087	19.1%
Q5	\$307,932	107,157	1,024	\$2,335,050	7.3%

Cost by county

County	Cost total	Prenatal	Postnatal	Recipients
Hawaii	\$4,609,449	\$1,452,292	\$3,157,157	2,021
Honolulu	\$21,932,790	\$6,910,331	\$15,022,459	9,614
Kauai	\$1,057,596	\$333,215	\$724,381	464
Maui	\$4,488,148	\$1,414,074	\$3,074,074	1,967

Both arms are driven by OBSERVED births (age-0 dependents in each PUMS tax unit), calibrated to CDC NVSR resident births and projected to the target year with the repo's damped-trend ensemble. Each eligible birth draws one prenatal + one postnatal payment. Cost = the take-up-adjusted benefit weighted by the household (WGTP) weight. Eligibility is tested on MAGI at the tax-unit (MAGI-household) grain, the family Medicaid uses. Benefits-by-quintile rank SPM units on summed income into population-equal fifths. Sampling CI is ACS sampling error (SDR, 80 replicate weights); the assumption band sweeps take-up and the birth count (+/-25%). 2026-2028 FPL is CPI-projected off the 2025 HHS table. Full methodology: RXKIDS_METHODODOLOGY.md.